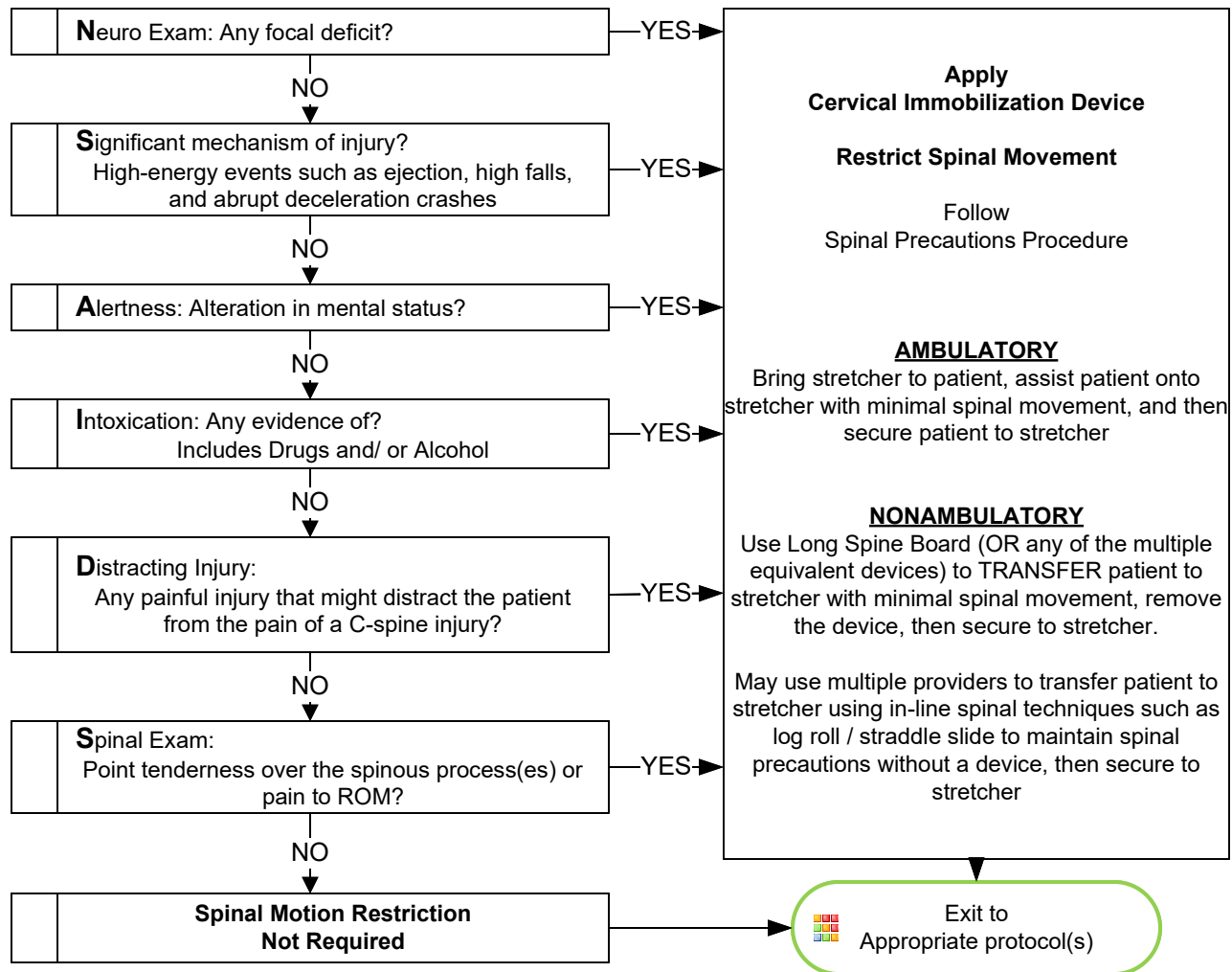




Selective Spinal Motion Restriction



Pearls

- **Recommended Exam: Mental Status, Skin, Neck, Heart, Lungs, Abdomen, Back, Extremities, Neuro**
- **Patients meeting all the above criteria do not require spinal motion restriction. However, patients who fail one or more criteria above require spinal motion restriction, but does NOT require use of the long spine board for immobilization.**
- **Long spine boards are NOT considered standard of care in most cases of potential spinal injury. Spinal motion restriction with cervical collar, and securing patient to cot, while padding all void areas is appropriate.**
- **True spinal immobilization is not possible. Spine protection and spinal motion restriction do not equal long spine board.**
- **Spinal motion restriction is always utilized in at-risk patients. This includes cervical collar, securing to stretcher, minimizing movement/ transfers. and maintenance of in-line spine stabilization during any necessary movement/ transfers. This includes the elderly, or others with body or spine habitus preventing them from lying flat.**
- **Consider spinal motion restriction in patients with arthritis, cancer, dialysis, and underlying spine or bone disease.**
- **Range of motion (ROM) is tested by touching chin to chest (look down), extending neck (look up), and turning head from side to side (chin to each shoulder) only in patients without posterior cervical mid-line pain. ROM should NOT be assessed if patient has midline spinal tenderness. Patient's range of motion should not be assisted, they must be able to complete alone.**
- **EMR may participate in spinal motion restriction per Agency Medical Director.**
- **Immobilization on a long spine board is not necessary where:**
 - Penetrating trauma to the head, neck or torso with no signs and/ or symptoms of spinal injury.
- **Concerning mechanisms that may result in spinal column injury:**
 - Fall from ≥ 3 feet and/ or ≥ 5 stairs or steps. Ground level falls in patients ≥ 65 years of age.
 - MVC ≥ 30 mph, rollover, and/or ejection
 - Motorcycle, bicycle, other mobile device, or pedestrian-vehicle crash
 - Diving or axial load to spine
 - Electric shock